CLOBETASOL

Very potent — short courses

Avoid face & skin folds

CHEMICAL NAME 21-chloro-9-fluoro-11β,17-dihydroxy-16β-methylpregna-1,4-diene-3,20-dione 17-propionate

CLASS Very potent topical corticosteroid (class I)

TRADE NAMES Dermovate · Clobex · Temovate · Etrivex (shampoo)

THERAPEUTIC USES / INDICATIONS

- Severe, resistant inflammatory dermatoses: chronic plaque psoriasis (limited areas), lichen planus, discoid lupus.
- Lichen sclerosus (genital) — first-line.
- Severe eczema not responding to less potent steroids (short-term).
- Scalp psoriasis (shampoo/scalp application).

ADMINISTRATION

- **Topical** thin layer 1–2 times daily using fingertip units (1 FTU ≈ 0.5 g covers two adult palms).
- Wash hands after (unless treating hands).
- No occlusive dressings unless instructed.
- Ointment for dry, thick skin; cream for moist areas.

DOSAGES

- **Adults:** apply once or twice daily for up to 4 weeks; max 50 g/week.
- **Lichen sclerosus:** once daily for 4 weeks, alternate days for 4 weeks, twice weekly for 4 weeks, then as needed.
- Not recommended in children <1 yr; caution in children.

PHARMACOKINETICS

- Absorption ↑ with inflamed skin, occlusion, thin skin (face, genitals, flexures), infants.
- Systemic absorption can suppress adrenal axis.

PHARMACODYNAMICS

- Potent **glucocorticoid receptor** agonist → anti-inflammatory, antipruritic, vasoconstrictive, antiproliferative effects in skin.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Burning, stinging.
- Skin thinning (atrophy), striae.
- Telangiectasia, folliculitis.

ADVERSE EFFECTS (SERIOUS)

- Skin atrophy, striae, purpura (prolonged/face/folds).
- Adrenal suppression, Cushing's (large areas, occlusion, children).
- Rebound flare/topical steroid withdrawal; pustular psoriasis on stopping.
- Perioral dermatitis, rosacea, glaucoma/cataract (periocular use).

CONTRAINDICATIONS & CAUTIONS

- Untreated bacterial, fungal or viral skin infections.
- Rosacea, acne, perioral dermatitis.
- Pruritus without inflammation; perianal/genital pruritus (except lichen sclerosus).
- Children <1 yr.

KEY INTERACTIONS

- Minimal (topical); systemic steroid effects add up with oral steroids.

NURSING MANAGEMENT

- Teach fingertip-unit dosing; record amount used per week.
- Avoid face, flexures, genitals unless specifically prescribed (lichen sclerosus).
- Inspect skin for atrophy and infection.
- Step down to weaker steroid as improves.

PATIENT EDUCATION

- Use a thin layer only on affected areas for the time advised.
- Do not use on the face unless told to.
- Do not stop suddenly after long use; taper as advised.
- Moisturise regularly (emollients first, wait 30 min).

KEY POINTS

- Very potent topical steroid.
- Lichen sclerosus first-line.
- Max 50 g/week, up to 4 weeks.
- Skin atrophy; avoid face and folds.